



Re: Gamification- Medical

From Sahil Singh <sahil.singh@autovrse.in>

Date Fri 10/2/2026 5:41 PM

To Ganesh Dommaraju <its2ganesh@gmail.com>; Nandan Kolar <nandan.kolar@autovrse.in>

Hi Ganesh,

Thanks for sharing this, adding [@Nandan Kolar](#) to the mail chain

Get [Outlook for iOS](#)

From: Ganesh Dommaraju <its2ganesh@gmail.com>

Sent: Friday, 02 October 2026 16:22:59

To: Sahil Singh <sahil.singh@autovrse.in>

Subject: Fwd: Gamification- Medical

You don't often get email from its2ganesh@gmail.com. [Learn why this is important](#)

Hi - Please see below and let me know if you have any questions this should give additional level of details for your proceed further for initial estimation of time and efforts (draft)

Core objective of the project is to build a gamification app/ web application changing the way one consumes the information to replace passive case-based learning, The participant (a specialist doctor) is dropped at an unknown point in the respiratory tree and must reconstruct the case rather than be told it (with the 3d pictures and expertise of the doctor the doctor should be able to identify (the business will provide these pictures/references).

Three layers in one : **anatomical orientation (lung → lobe → segment → structure), pathophysiological interpretation (inflammation, mucus, remodeling, alveolar destruction, histology), and diagnostic reasoning** (asthma vs. COPD vs. exacerbation).at its core -> observe · orient · interpret · decide —

Clues to find :

Where am i : Right or left lung, lobe, segment, and structure: trachea, carina, bronchus, bronchiole, alveolus, etc.

What am I seeing: Airway diameter, branching, inflammation, mucus, thickening, remodeling, cellular infiltration, or alveolar destruction.

What does Patient Have : Interpret the evidence and define whether the scenario corresponds to asthma, COPD, exacerbation, or another clinical condition

Clues to Solve the Case

VISUALS:Inflammation · mucus · remodeling

ANATOMICAL:Orientation · diameter · branching

HISTOLOGICAL: Epithelium · infiltration · destruction

BIOMARKERS: Eosinophils · FeNO · others

CLINICAL AUDIO :Wheezing · rhonchi · breath sounds

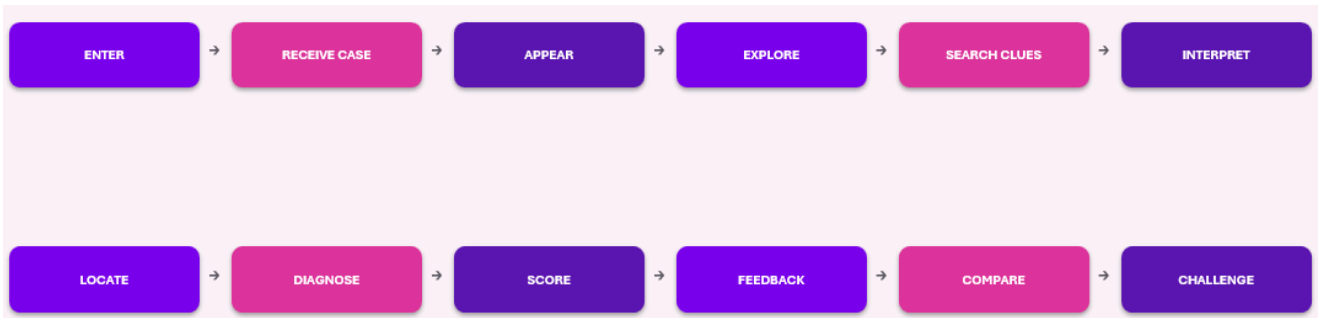
Scoring should be on -> anatomical precision + diagnostic precision + speed, duels, Phase 1 (respiratory) is a proof of a modular architecture: same engine, new organs and pathologies in Phase 2 (dermatology, cardiovascular, neurology, gastroenterology)

there should be levels : like basic level, intermediate and advanced in the game (this can be little later)

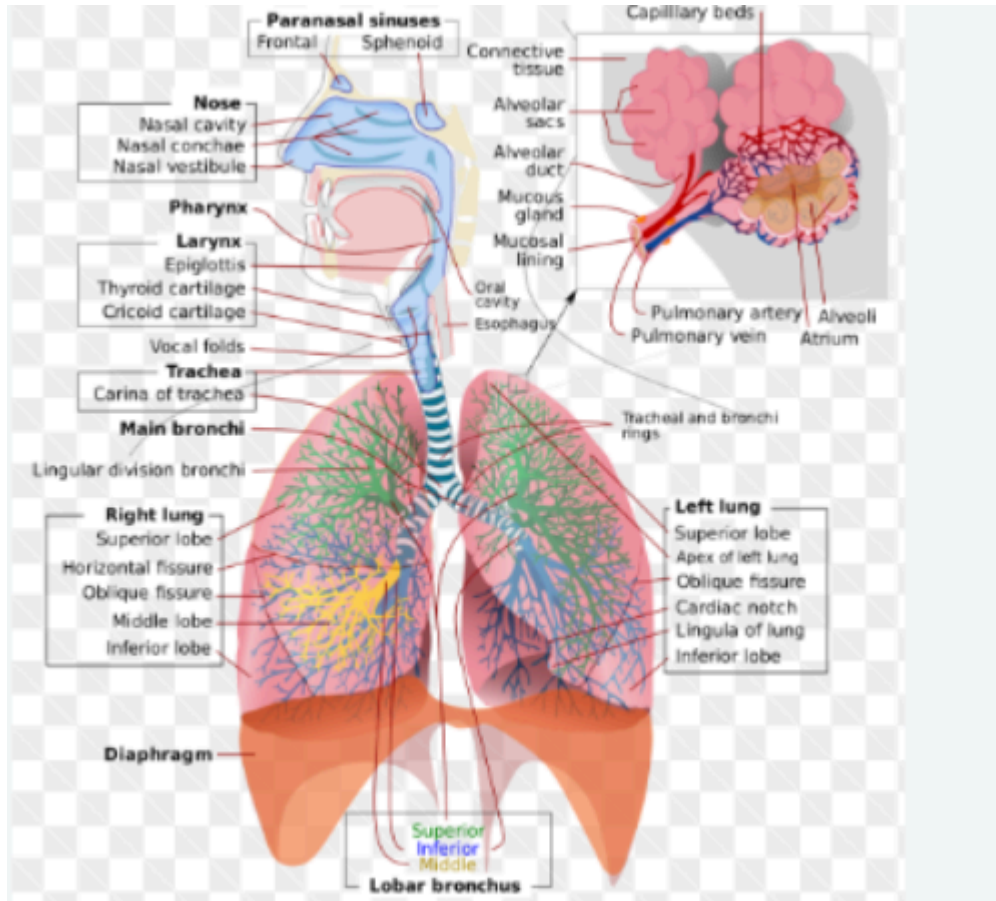
Five components of technical structure should be

Component	What it is	approach
3D environment	Navigable respiratory anatomy, from trachea to alveoli	anatomically validated lung model segmented by lobe/segment with tagged structures (trachea, carina, bronchus, bronchiole, alveolus).needs more details from expert business team.
Case engine	Scenarios + clue sets	Aase = patient profile + location node + visual/histological/biomarker/audio clue layers + correct answers.
Game engine	Rules, modes, scoring	difficulty tiers (basic → expert), scoring = anatomical placement + diagnostic correctness + time.
Multiplayer	Live and asynchronous	Replay stored rounds. Live duels and KOL(experts) challenge layer on the same round-record format.
Feedback + data	Explanation engine and analytics	Every wrong answer maps to the specific clue that should have been read; analytics feed profile, evolution by level, and rankings by country/specialty/congress/institution.

End to end process:



reference picture(from Google images)



Game: 3 core layers



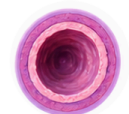
MARK LOCATION

Lung → lobe → segment



SELECT STRUCTURE

Bronchus · bronchiole · alveolus...



DEFINE DIAGNOSIS